

Overview of the US Healthcare System

The US Healthcare System is unique globally. It is neither a single-payer/universal healthcare system like those in the UK or Canada, nor does it have unified national pricing for medical services.

Instead, it operates as a **hybrid system** dominated by the private market and supplemented by public government programs.

1. Major Sources of Health Insurance Coverage

Because healthcare in the US is extremely expensive and nearly impossible to afford out-of-pocket, the vast majority of residents rely on health insurance obtained through the following channels:

- **Employer-Sponsored Insurance (ESI):** Covers approximately 50% of the population. Most employed individuals receive group health plans through their employers, where the employer subsidizes the majority of the premium and the employee pays the remainder monthly.
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- **Medicare:** A federal public insurance program primarily designed for seniors aged 65 and older, as well as younger individuals with end-stage renal disease (ESRD) or specific qualifying disabilities.
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- **Medicaid & CHIP:** Joint federal and state programs that offer low-cost or free coverage to low-income individuals, families, pregnant women, children (via Children's Health Insurance Program), and elderly individuals needing long-term care.
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- **Obamacare / Health Insurance Marketplace:** Designed for freelancers, self-employed individuals, or those without access to employer coverage. Income-based subsidies (Premium Tax Credits) are provided by the government to lower costs.
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- **Military & Veterans Affairs (TRICARE / VA):** Government-funded programs providing healthcare coverage to active-duty military personnel, veterans, and their dependents.
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2. Service Providers & Network Mechanisms

The vast majority of healthcare providers, clinics, and hospitals in the US are private, for-profit or non-profit entities. Insurance companies partner with these providers through contractual **Medical Networks**:

- **In-Network:** Providers who have signed discounted rate agreements with the insurer. Using in-network care guarantees higher coverage levels and significantly lower out-of-pocket costs.
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- **Out-of-Network:** Providers without contractual agreements with your insurer. Coverage is minimal or non-existent, leaving patients exposed to full fees or "balance billing."
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Common Plan Structures

- **HMO (Health Maintenance Organization):** Lower premiums, but requires choosing a Primary Care Physician (PCP). Specialist visits strictly require a PCP referral, and out-of-network care is not covered (except for emergencies).
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- **PPO (Preferred Provider Organization):** Higher premiums, but maximum flexibility. No PCP referrals are needed to see specialists, and out-of-network care receives partial coverage.
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- **EPO (Exclusive Provider Organization):** Moderate premiums; no referrals needed for specialists, but offers zero coverage for out-of-network care.
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3. Essential Cost-Sharing Concepts

Beyond the monthly **Premium** (which does not count toward cost-sharing limits), patients share medical expenses with insurers through the following mechanisms:

Term	Description & Mechanism
Deductible	The fixed dollar amount you must pay 100% out-of-pocket each year before your insurance coverage kicks in (preventive care is usually exempt).
Copay	A fixed flat fee paid at the time of service or prescription pickup (e.g., \$20 per PCP visit or \$15 per generic drug).
Coinsurance	The cost-sharing percentage split between you and your insurer after meeting your deductible (e.g., insurer covers 80%, you pay 20%).

Out-of-Pocket Maximum (OOPM)	The absolute maximum cap on what you pay for covered in-network services in a plan year. Once reached, insurance covers 100% of eligible expenses.
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